

Long-term cognitive deficits in memory, attention, and executive functioning in post-COVID-19 syndrome patients

Kozik, V.¹, Reuken, P.², Utech, I.¹, Gramlich, J.², Stallmach, Z.², Demeyere, N.³, Schwab, M.¹, Rakers, F.¹, Stallmach, A.^{2,4}, Finke, K.^{1,5}

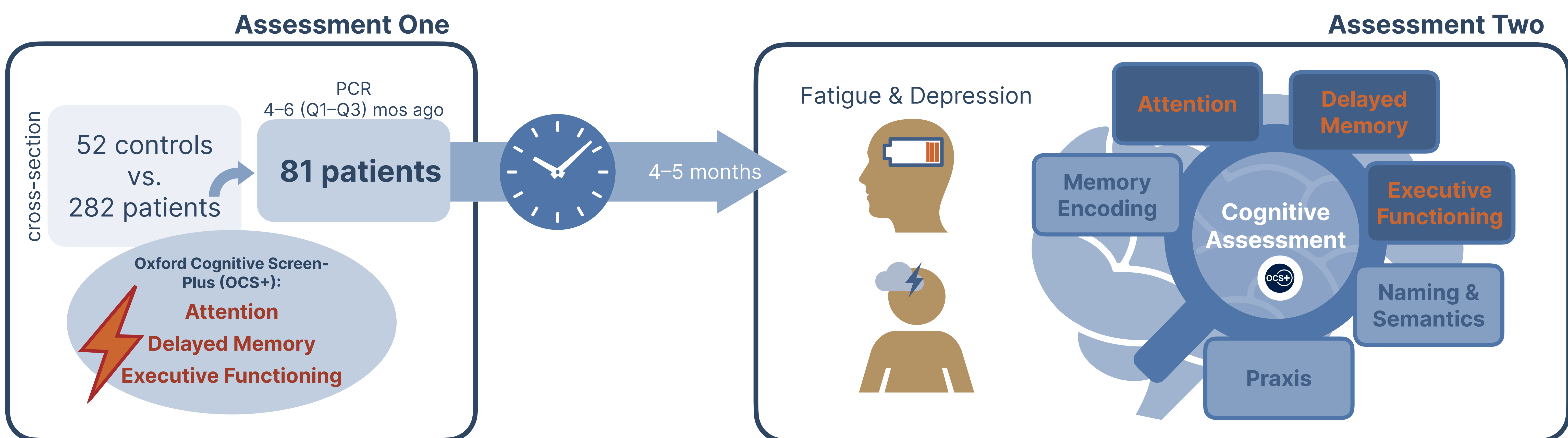
¹Department of Neurology, University Hospital Jena, Germany; ²Department of Internal Medicine IV (Gastroenterology, Hepatology, and Infectious Diseases), University Hospital Jena, Germany; ³Department of Clinical Neurosciences, University of Oxford, Oxford, UK; ⁴Center for Sepsis Control and Care (CSCC), University Hospital Jena, Germany; ⁵Department of Psychology, Ludwig Maximilian University of Munich, Germany

Background & Methods

- Previous study: **deficits in attention, delayed verbal memory, and executive functioning** in comparison to healthy controls (Kozik et al., 2023)
- Cross-sectional analysis: no effect of the time since infection → first indication of symptom persistence

Important question: How do these cognitive deficits change over time?

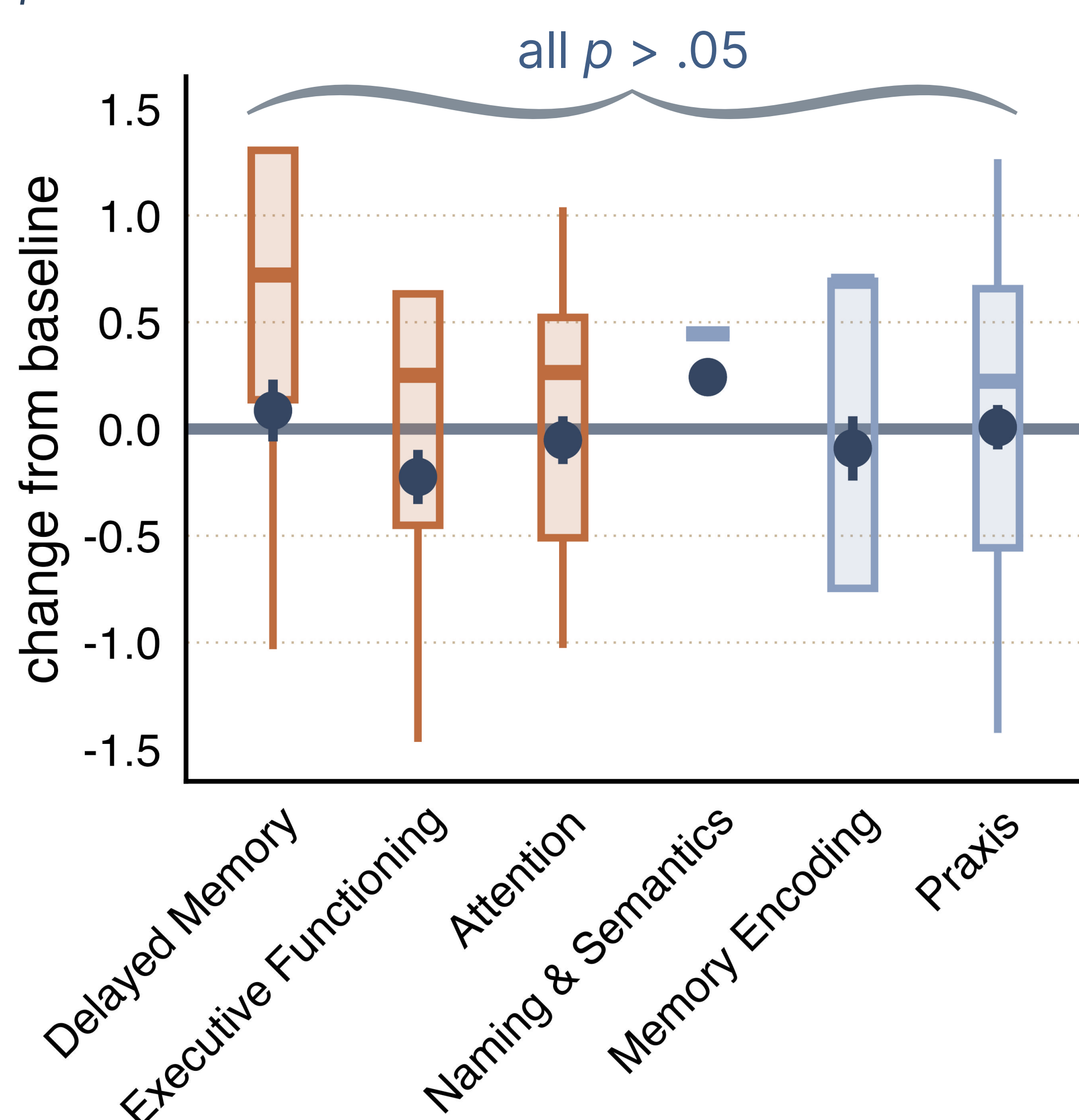
Longitudinal analysis: re-examination of **81 patients** previously assessed 5 months (median) after infection



Results

Cognitive Assessment

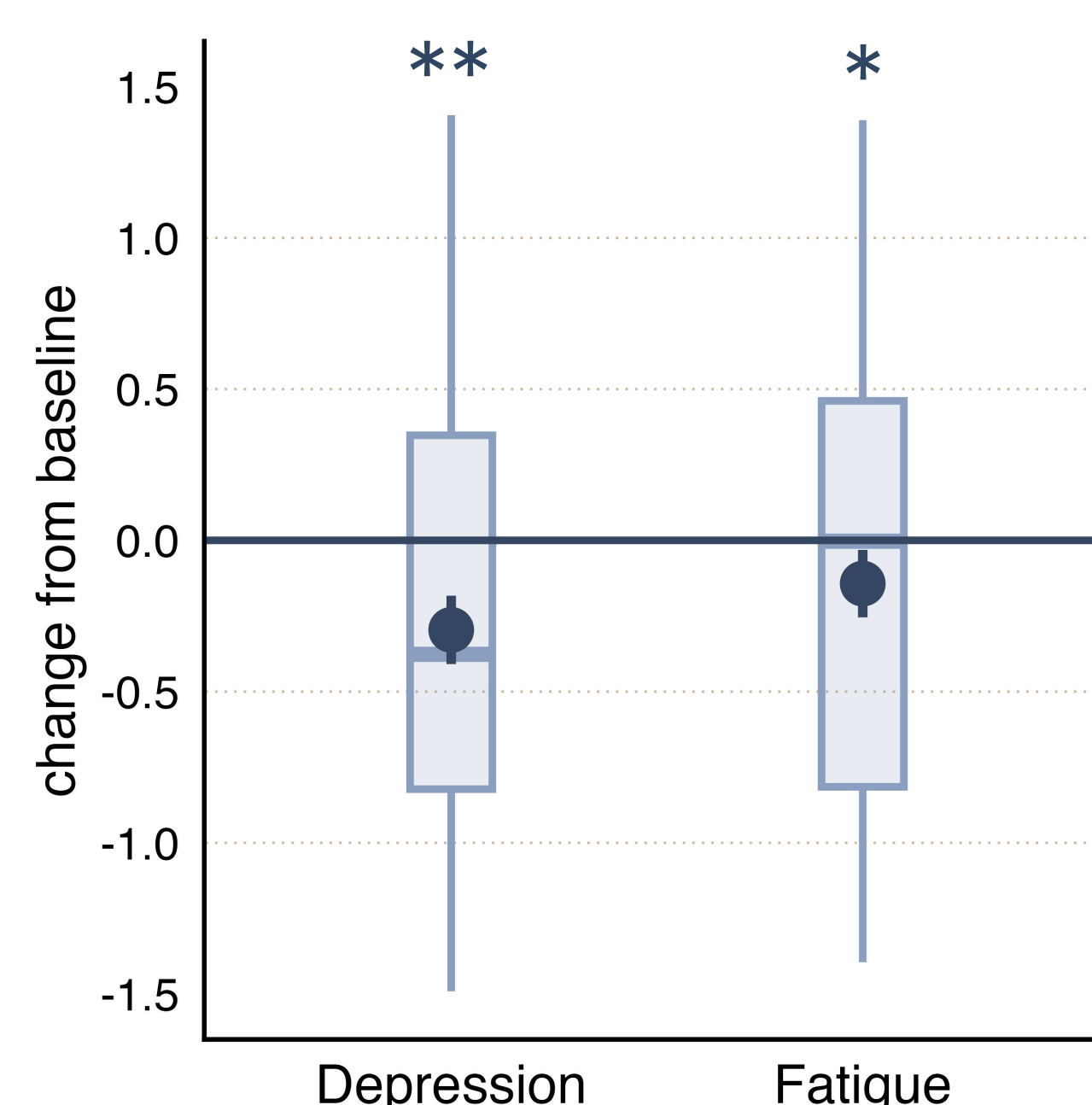
- ◆ **No observed changes in any cognitive domain** (all $p > .05$)
- ◆ Small positive tendency (n.s.) in delayed verbal memory ($V = 820$, $HL = .5$, $95\% \text{ CI } [-.5, 1]$, $p = .35$, Yuen's t-test $t(64) = 1.7$, $p = .09$)



Note. Displayed are z-scores of cognitive performance at assessment two, as compared to baseline (line at zero). Point ranges denote mean and standard error of the mean (SEM).

Fatigue and Depression

- ◆ Patients reported **lower levels of depression** ($V = 719$, $HL = -1.5$, $95\% \text{ CI } [-2.5, -0.5]$, $p < .01$).
- ◆ Patients' reports of **fatigue severity tended to be lower** ($V = 866.5$, $HL = -2$, $95\% \text{ CI } [-3.5, <0]$, $p = .041$).



Note. Displayed are z-scores of depression and fatigue at assessment two, as compared to baseline (line at zero). Point ranges denote mean and standard error of the mean (SEM).

Abbreviations:
 Q1, 3 = quartiles 1, 3;
 HL = Hodges-Lehmann estimator, a robust measure of location (median of all possible pairwise differences between observations);
 ** $p < .01$, * $p < .05$

Conclusions

- ◆ The **profile of neurocognitive deficits remained stable**, in quality and quantity: no observed changes in any cognitive domain
- ◆ Lower levels of self-reported **symptoms of depression and fatigue severity**
- ◆ This follow-up after early first assessment offers evidence for the **stability of neurocognitive deficits** and their potentially **chronic course**.

